



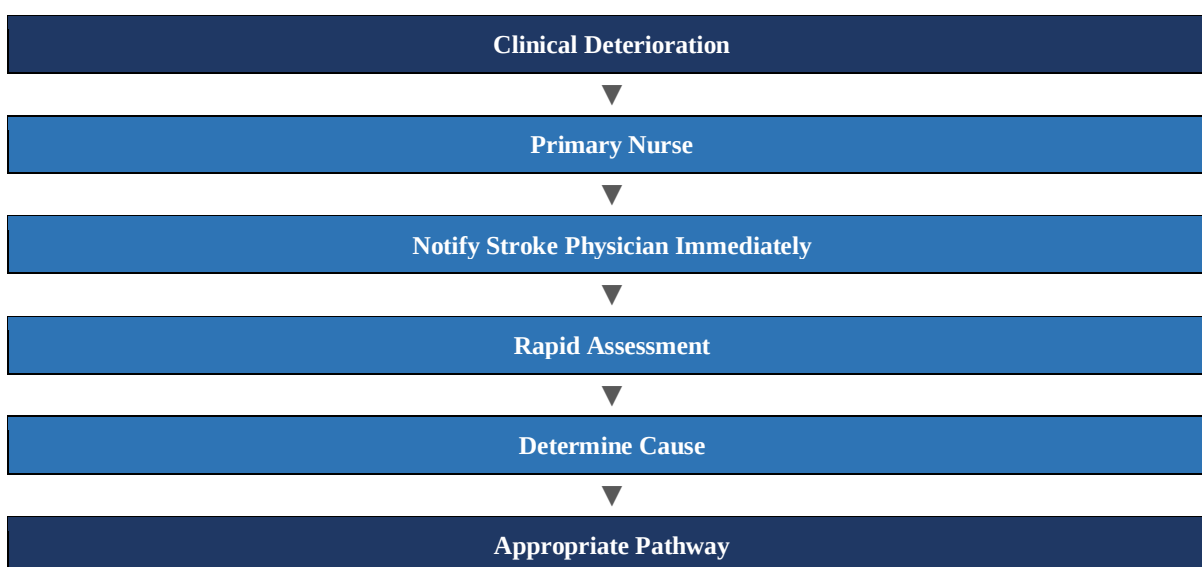
Stroke Patient Escalation Protocol

Stroke Program — Clinical Escalation

Purpose

To provide a standardized response when a stroke patient clinically deteriorates.

General Escalation Flow



Deterioration-Specific Pathways

Neurological Deterioration

Examples: NIHSS increase ≥ 4 · GCS decline · new weakness · new aphasia

- Immediate physician review
- STAT CT Brain
- Consider CTA
- Notify neurosurgery
- ICU transfer if needed

Hemorrhagic Transformation

- Stop thrombolytic/anticoagulant if applicable
- Repeat CT
- Reverse anticoagulation if indicated
- Neurosurgery consult
- ICU admission

Airway Compromise

- Respiratory therapy
- Anesthesia
- Intubation
- ICU

Seizures

- Notify stroke physician
- Lorazepam
- Antiepileptic loading
- EEG
- Neurology review

Blood Pressure Crisis

- Follow IV antihypertensive protocol
- Frequent monitoring
- Escalate to ICU if uncontrolled

Cerebral Edema

- Repeat CT
- Hyperosmolar therapy
- Neurosurgery consult
- ICU

Cardiac Complications

- ECG
- Troponin
- Cardiology consult
- Telemetry

Documentation

All escalations must be documented in:

- Progress notes
- Stroke flowsheet
- Escalation record
- ICU transfer note (if applicable)

AHA/ASA Guideline Alignment & Accreditation

Guideline concordance: This escalation protocol operationalizes the neurological monitoring, hemorrhagic transformation, and blood pressure management principles of the 2026 AHA/ASA Guideline for the Early Management of Patients with Acute Ischemic Stroke, ensuring bedside deterioration triggers a guideline-concordant, physician-led response.

Accreditation support: Defined escalation triggers, timed pathways (STAT CT, neurosurgery/ICU notification), and mandatory documentation in the stroke flowsheet and escalation record provide the auditable evidence trail required for Comprehensive Stroke Center (CSC) certification (The Joint Commission / DNV Advanced Certification) and AHA Get With The Guidelines®-Stroke performance reporting.

Approval & Sign-Off

Role	Name & Title	Signature	Date
Prepared By			
Reviewed By (Stroke Coordinator)			
Approved By (Stroke Medical Director)			
Approved By (Chief Nursing Officer / Nursing Director)			

Effective date: _____ Next review date: _____